

LifeLink AI

Business Model & Revenue Strategy

"An affordable wearable, a sustainable subscription engine."



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LifeLink AI is built to be affordable on day one and profitable over time: a low-margin hardware entry point paired with recurring subscription and institutional-licensing revenue that grows as trust and adoption grow.

01 Executive Summary

LifeLink AI turns a low-cost QR medical bracelet into a durable revenue engine. The bracelet itself is sold near cost to remove any barrier to adoption; the business is designed to earn recurring revenue from cloud-hosted subscriptions, institutional partnerships, and hospital/EMS software licensing — the same pattern used by razor-and-blade and freemium SaaS models, applied to emergency healthcare.

Business Goal: Reach sustainable, recurring revenue by pairing an affordable hardware entry point with high-retention institutional and subscription income — without pricing any individual wearer out of basic protection.

Low

one-time hardware cost, priced near production cost

Free

basic emergency profile for every wearer, always

Recurring

premium subscriptions and institutional licensing

B2B2C

universities, employers, and insurers sponsor bracelets at scale

02 Market Opportunity

Bangladesh alone records over a hundred thousand road-traffic casualties a year alongside a large and growing population living with chronic conditions such as diabetes, cardiovascular disease, and epilepsy — a sizeable addressable base for an affordable emergency-identity wearable. This funnel, from total population at risk down to a realistic first-year pilot cohort, frames the near-term commercial opportunity.

Market Opportunity — TAM / SAM / SOM

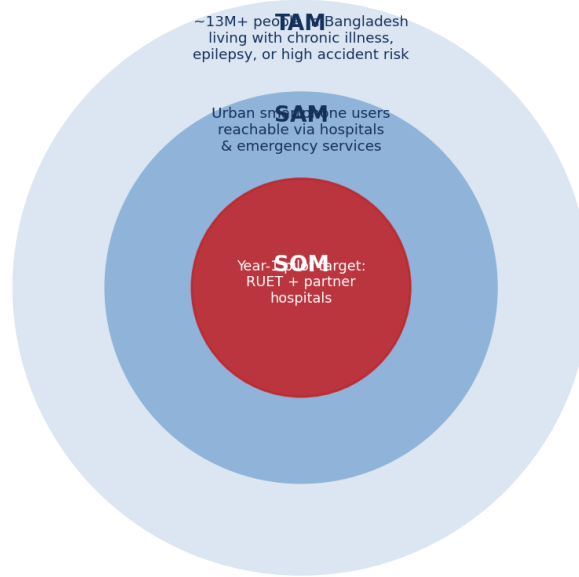


Figure 1: Market sizing: from the total population at risk (TAM) to a realistic Year-1 pilot cohort (SOM).

Layer	Commercial Read
TAM — Total population at risk	Everyone living with a chronic condition or regular road exposure; the long-run ceiling for hardware and subscription revenue.
SAM — Serviceable market	Urban and campus populations reachable through hospital, university, and employer partnerships within the current distribution model.
SOM — Obtainable market	A realistic Year-1 pilot cohort acquired through a small number of anchor institutional partners, used to prove retention and unit economics before wider rollout.

03 Customer Segments

The business model rests on two linked customer groups: individual wearers who need protection, and institutions that pay to provide or license it at scale.

Individual Wearers (B2C)

- Road accident-prone commuters
- Elderly individuals
- Heart, stroke, diabetes, and epilepsy patients
- Children, students, and people with severe allergies

Price-sensitive; converted mainly through the free tier and low-cost bracelet.

Institutional Buyers (B2B2C)

- Universities and student welfare programs
- Corporate employers and workplace safety programs
- Insurers offering the bracelet as a policy add-on
- Hospitals and ambulance/EMS services

Higher willingness to pay; the primary source of recurring, contract-based revenue.

04 Value Proposition

For wearers — a free, always-on safety net that costs nothing beyond an affordable bracelet, with optional upgrades for families and travelers.

For institutions — a low-cost way to demonstrably improve emergency response for members, students, or employees, with usage data and audit logs to justify the spend.

For hospitals & EMS — a dashboard that shortens time-to-information on arrival, licensed as recurring software revenue rather than one-off hardware.

For LifeLink AI — a diversified revenue base — individual, institutional, and clinical — that reduces dependence on any single customer type.

05 Competitive Positioning

Compared with static engraved medical-ID jewelry and generic health-record apps, LifeLink AI is designed to win on the dimensions that matter most in an emergency — speed, zero-friction access, and AI-prioritized information — while remaining as affordable as a traditional ID bracelet. That combination is also what supports a subscription and licensing model: static competitors have no recurring service to sell, while app-only competitors lack a durable physical anchor.

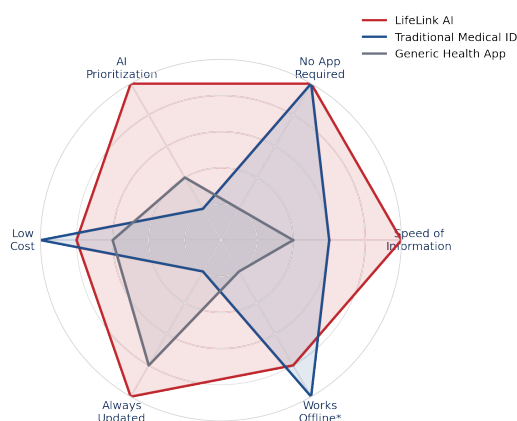


Figure 2: LifeLink AI vs. traditional medical ID vs. generic health apps, rated 1 (low) –5 (high) per dimension. *Offline access refers to the bracelet always being physically present, even where connectivity is briefly unavailable to the wearer.

06 Revenue Streams

How LifeLink AI Earns

- **One-time hardware sale** — QR bracelet, sold at low margin to maximize adoption.
- **Freemium profile** — free basic emergency profile for every wearer.
- **Premium subscription** — advanced AI summaries, family sharing, multi-language briefs, priority support.
- **Institutional licensing** — universities, corporates, and insurers sponsor bracelets for members/employees.
- **Hospital/EMS dashboard licensing** — subscription for ambulance services and hospital emergency departments.

Why It's Sustainable

The core product requires no proprietary hardware beyond an inexpensive QR bracelet, so unit costs stay low. Recurring revenue comes from cloud subscriptions and institutional partnerships rather than the hardware itself, which keeps the wearable affordable for individual buyers while building durable relationships with hospitals and universities.

The hardware sale is a customer-acquisition cost, not a profit center — margin is designed to come from the services layered on top of it.

07 Pricing Strategy

Pricing is tiered so that no wearer is priced out of basic protection, while institutions and heavier users fund the majority of recurring revenue.

Tier	Who It's For	What's Included
Free	Every individual wearer	Basic emergency profile, one AI summary, standard QR bracelet at cost
Premium	Individuals & families wanting more	Family sharing, multi-language AI briefs, priority support, profile history
Institutional	Universities, employers, insurers	Bulk-priced bracelets, member dashboards, usage reporting, dedicated onboarding
Hospital / EMS	Hospitals & ambulance services	Responder dashboard, integration support, service-level agreement, audit logs

The free tier is not a loss leader to be phased out — it is the trust-building core of the model. Every paid tier sits on top of it rather than replacing it.

08 Cost Structure

Built on technology that exists today: QR codes, cloud servers, a mobile application, a secure database, and an AI summarization module. Because no specialized hardware is required, deployment costs remain relatively low while allowing the system to scale for hospitals, ambulance services, universities, and government healthcare programs.

Estimated Pilot-Phase Budget Allocation

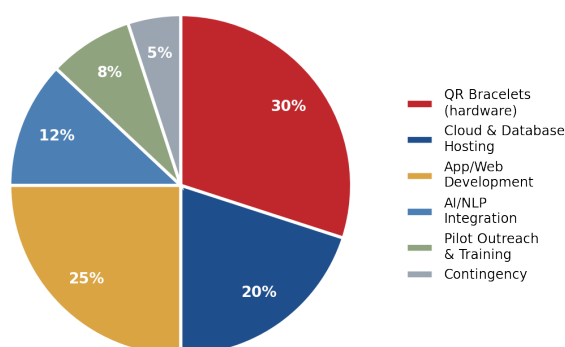


Figure 3: Indicative budget allocation for a 6-month campus + partner-hospital pilot.

Fixed / Semi-Fixed Costs

- Cloud hosting and database infrastructure
- AI summarization module (development & inference)
- Core engineering and product team
- Compliance, security, and audit tooling

Variable Costs

- QR bracelet manufacturing and fulfillment
- Per-wearer cloud storage and AI inference at scale
- Hospital/EMS onboarding and support
- Institutional account management

09 Unit Economics

Because the bracelet is sold near cost, the model's health depends on converting a share of free-tier wearers to premium, and on winning multi-year institutional contracts that carry low marginal cost per additional member.

Low CAC per institutional member — one partnership conversation can activate hundreds or thousands of wearers at once, versus one-by-one consumer acquisition.

High retention by design — the bracelet never needs replacing and profile edits apply instantly, so there is little reason for an institution or premium subscriber to churn.

Marginal cost stays low — cloud storage and AI inference costs per wearer are small relative to subscription and licensing fees.

Hardware is cost-neutral, not profit-driving — bracelet margin is kept thin on purpose — profitability is designed to come from the services layered above it.

At this stage the model is directional rather than audited: real CAC, LTV, and churn figures will be established during the pilot phase described in Section 14.

10 Go-to-Market Strategy

Phase 1 — Anchor Trust

1. Partner with 1-2 local hospitals and ambulance services for credibility and real-world validation.
2. Launch a university pilot to reach a concentrated, digitally engaged first cohort.
3. Use free-tier adoption to build the wearer base before pushing premium upsell.

Phase 2 — Scale Through Institutions

1. Convert pilot results into case studies for employer and insurer partnerships.
2. Sell hospital/EMS dashboard licensing as a standalone recurring product.
3. Expand premium subscription features to grow individual-tier revenue.

11 Key Partnerships & Channels

Partner Type	Role in the Business Model
Universities	Distribution channel for the founding pilot cohort; potential institutional licensee for student welfare programs.
Hospitals & ambulance services	Clinical validation partner and paying customer for the responder dashboard.
Corporate employers	Institutional buyer sponsoring bracelets as a workplace safety or wellness benefit.
Insurers	Potential distribution partner offering the bracelet as a policy add-on, tying LifeLink AI to an existing customer relationship.
Government health programs	Longer-term channel for national-scale distribution and interoperability.

12 Financial Sustainability

LifeLink AI does not claim to prevent every negative outcome. Instead, it is designed to remove one specific, solvable bottleneck — the time and uncertainty involved in accessing a patient’s emergency medical history — and to fund that mission through revenue streams that scale independently of hardware margin.

Diversified

revenue across consumer, institutional, and clinical customers

Recurring-first

subscriptions and licensing, not one-time hardware, carry the model

Low marginal cost

cloud-native architecture scales without proportional cost growth

13 Risks to the Business Model

Risk	Mitigation
Low conversion from free to premium tier	Keep premium value clearly differentiated (family sharing, multi-language briefs) rather than gating core safety features
Slow institutional sales cycles	Start with a university + partner-hospital pilot to build trust and evidence before wider enterprise sales
Data privacy or breach concerns undermining institutional trust	End-to-end encryption, role-based access, full audit logs, compliance-ready architecture
Hardware margin pressure at scale	Keep bracelet cost-neutral by design; protect margin in the subscription and licensing layers instead
Responder unfamiliarity limiting perceived institutional value	No app required to scan — any smartphone camera works; simple onboarding for hospitals/EMS

14 Growth Roadmap

From campus pilot to national emergency infrastructure — each phase is designed to unlock the next revenue layer.

Phase 1 · 0-6 months Pilot Launch

- QR bracelet + mobile app for early adopters
- Free-tier adoption to build the wearer base
- Partner with 1-2 local hospitals & ambulance services
- Validate willingness to pay for premium features

Phase 2 · 6-18 months Regional Scale-Up

- Hospital dashboard & EMS integration licensing
- First institutional (university/employer) contracts
- Multi-language AI summaries drive premium upgrades
- Government & university health-program partnerships

Phase 3 · 18+ months National & Beyond

- Insurer distribution partnerships at scale
- NFC-enabled smart bracelet & wearable integration
- National health database interoperability
- International traveler and cross-border licensing

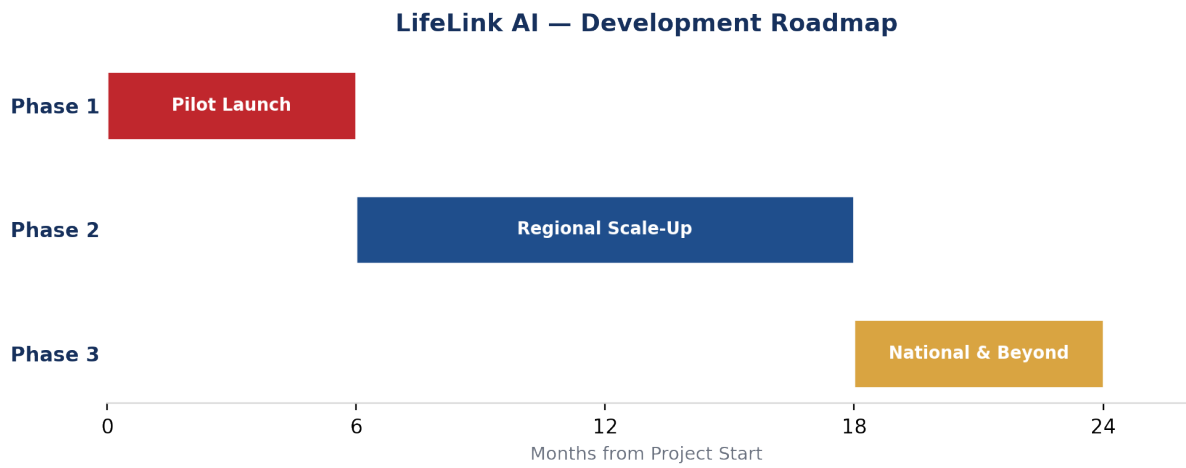


Figure 4: Development timeline across the three roadmap phases.

15 Conclusion

LifeLink AI's business model is deliberately layered: a low-margin, high-adoption hardware entry point; a free tier that builds trust and a wearer base; and recurring, higher-margin revenue from premium subscriptions and institutional and clinical licensing. This structure keeps the core safety product affordable for individuals while giving the business a durable, scalable path to sustainability.

The bracelet earns adoption. The subscriptions and institutional contracts earn the business. LifeLink AI is designed so neither has to compromise the other.

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Thank you for considering LifeLink AI.